UNIVERSITY OF SOUTHEASTERN PHILIPPINES
SCHOOL OF MEDICINE
Apokon, Tagum City, Philippines

COMPREHENSIVE HEALTH HISTORY TAKING

Case Scenario: Abdominal Pain in a 42 Year Old Male

A Written Report Presented to the
Faculty of School of Medicine

In Partial Fulfillment
of the Requirements for the Physical
Diagnosis of Doctor of Medicine II

LABASANO, ZANDRO JUSTINE F.

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I. INTRODUCTION

This activity aims to develop competency in performing a comprehensive health history and focused physical examination through patient-centered communication, systematic questioning, and accurate clinical assessment. Through this case, the student will practice eliciting the characteristics of headache, identifying associated symptoms and possible triggers, assessing relevant medical and family history, and performing a focused neurological examination.The activity emphasizes the application of essential Physical Diagnosis concepts, including inspection, palpation, assessment of vital signs, examination of cranial nerves, motor and sensory function, coordination, and evaluation for meningeal irritation.

II. CASE SCENARIO

Headache in a 28-year-old female

The patient is a 28-year-old female who presents with a headache that began two days ago. Initially mild, the headache has progressively worsened and is primarily located in the frontal and temporal regions, with mild occipital discomfort.

The pain is constant and described as throbbing, accompanied by a pressure-like sensation, particularly over the frontal region. It is aggravated by bright lights, loud sounds, physical activity, and work-related stress. Resting in a dark, quiet room provides temporary relief, although the headache returns upon resuming normal activities.

The headache is associated with nausea, photophobia, and phonophobia, without vomiting. The patient denies aura, visual disturbances, and weakness. She rates the pain as 8/10 in intensity.

The patient has no known chronic medical conditions and no history of previous surgery. She occasionally takes ibuprofen for headaches and menstrual cramps. She has no known allergies. Her mother has a history of migraine beginning in her 30s, while her father and siblings have no significant medical conditions.

The patient works as a graphic designer, spending long hours in front of a computer. She has a predominantly sedentary lifestyle and rarely exercises. She does not smoke and occasionally consumes alcohol, approximately 1–2 drinks per week. She reports having a balanced diet and no identified food triggers.

Review of Systems:

Generally unremarkable except for headache, nausea, photophobia, and phonophobia.

III. COMPREHENSIVE HEALTH HISTORY

A. Identifying Information

Particulars	Information
Name	To be obtained
Age	28 years old
Sex	Female
Address	To be obtained
Civil Status	To be obtained
Religion	To be obtained
Occupation	Graphic Designer

Date of Consultation To be obtained

B. Chief Complaint
"Matindi po ang sakit ng ulo ko, Doc." (Severe headache)

C. History of Present Illness
The patient is a 28-year-old female who presents with a two-day history of progressively worsening headache. The headache initially started as mild discomfort but gradually intensified to 8/10 in severity.
The pain is primarily localized over the frontal and temporal regions, with mild discomfort in the occipital region. It is constant and described as throbbing, with an accompanying pressure-like sensation, particularly in the frontal area.
The headache is aggravated by exposure to bright lights, loud sounds, physical activity, and work-related stress. Temporary relief is experienced when resting in a dark and quiet room, although the headache returns when normal activities are resumed.
Associated symptoms include nausea, photophobia, and phonophobia. The patient denies vomiting, aura, visual disturbances, and focal weakness.
There is a positive family history of migraine, with the patient's mother experiencing similar headaches beginning in her 30s.
The patient occasionally takes ibuprofen for headaches and menstrual cramps. Further clarification regarding the dose, frequency, and response to ibuprofen is necessary.
Clinical relevance:
The headache characteristics suggest a migraine-type headache, particularly because of its throbbing quality, aggravation by physical activity, nausea, photophobia, and phonophobia. The positive maternal history further supports this possibility.
The duration of two days is compatible with a migraine attack. However, secondary causes of headache must still be considered and excluded through appropriate history-taking and physical examination.

D. Past Medical History
- Childhood Illnesses: Not specified; further history required.
- Medical Illnesses: No known chronic medical conditions.
- Hospitalizations: Not specified.
- Surgeries: No previous surgical procedures.
- Psychiatric History: Not specified.
- OB-Gyne History: Menstrual cramps are reported. Menstrual history, pregnancy possibility, and hormonal contraceptive use should be assessed.
- Medications: Occasional ibuprofen for headaches and menstrual cramps.
- Allergies: No known allergies.
- Health Maintenance: Not specified.
- Cancer History: Not specified.
- Genetic/Hereditary Diseases: No known personal history; relevant family history includes maternal migraine.

E. Family History
- Mother: History of migraine beginning in her 30s.
- Father: No significant known medical conditions.
- Siblings: No significant known medical conditions.

Relevant finding: Positive family history of migraine, which may indicate a familial predisposition to migraine headaches.

F. Personal and Social History

Category	Patient Information
Occupation	Graphic designer
Work Environment	Long hours working in front of a computer
Lifestyle	Predominantly sedentary
Diet	Balanced diet; no identified food triggers
Exercise	Rarely exercises
Smoking	Does not smoke
Alcohol Intake	Approximately 1–2 drinks per week
Medication Use	Occasional ibuprofen
Stress	Work-related stress aggravates headache
Sleep Pattern	Not specified; should be assessed
Hobbies/Interests	Not specified
Source of Support	Not specified
Coping Mechanism	Resting in a dark, quiet room provides temporary relief
Health Concerns	Persistent and progressively worsening headache

G. Review of Systems

System	Relevant findings
General/Constitutional	No other symptoms reported. Ask about fever, fatigue, and unexplained weight loss.
Skin	No reported abnormalities.
HEENT	Headache involving frontal, temporal, and mild occipital regions. Photophobia and phonophobia present. No reported visual disturbances.
Cardiovascular	No reported abnormalities; ask about chest pain, palpitations, and syncope.

Respiratory	No reported abnormalities.
Gastrointestinal	Nausea present; no vomiting.
Genitourinary	No reported abnormalities.
Musculoskeletal	No reported abnormalities; assess neck pain and stiffness.
Neurologic	Headache present. No aura or focal weakness. Ask about numbness, seizures, dizziness, confusion, and loss of consciousness.
Endocrine	No reported abnormalities.
Psychiatric	Work-related stress aggravates the headache.
Hematologic	No reported abnormalities.
Immunologic	No known allergies.

IV. SUMMARY OF FINDINGS

The patient is a 28-year-old female presenting with a progressively worsening, constant headache of two days' duration, primarily involving the frontal and temporal regions, with mild occipital discomfort.

The headache is described as throbbing and pressure-like, with an intensity of 8/10. It is aggravated by bright lights, loud sounds, physical activity, and work-related stress. Associated symptoms include nausea, photophobia, and phonophobia. There is no reported aura, visual disturbance, or focal weakness.

The patient's medical history is unremarkable for known chronic illnesses and previous surgeries. She occasionally uses ibuprofen for headaches and menstrual cramps. Her family history is significant for maternal migraine.

Her lifestyle is predominantly sedentary because of prolonged computer work, and she rarely exercises. She does not smoke and occasionally consumes alcohol.

The clinical presentation is suggestive of migraine without aura. The combination of headache characteristics, associated symptoms, aggravation by physical activity, and positive family history supports this impression.

Nevertheless, further assessment, including vital signs, neurological examination, fundoscopic examination, and evaluation for secondary headache warning signs, is necessary to establish the clinical impression and determine whether additional investigations are indicated.